

1. Administrative & Study Identification

Official Title: A Phase 3, Randomized, Open Label Study to Compare Nivolumab Plus Concurrent Chemoradiotherapy (cCRT) Followed by Nivolumab Plus Ipilimumab or Nivolumab Plus cCRT Followed by Nivolumab Versus cCRT Followed by Durvalumab in Previously Untreated, Locally Advanced Stage III Non-small Cell Lung Cancer (NSCLC) **Brief Study Title:** A Study of Nivolumab and Ipilimumab in Untreated Participants With Stage 3 Non-small Cell Lung Cancer (NSCLC) That is Unable or Not Planned to be Removed by Surgery **Short Title/Acronym:** CheckMate 73L **Protocol Number:** CA209-73L **NCT Number:** NCT04026412 **Company / Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab (Opdivo) and Ipilimumab (Yervoy) **Phase of Development:** Phase 3 (PHASE3) **Trial Status:** COMPLETED (Study has been completed) **Medical Monitor:** Joseph Fiore (VP, Global Program Lead, Thoracic Cancers, Bristol Myers Squibb) / BMS Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effectiveness, specifically Progression-Free Survival (PFS), of nivolumab plus concurrent chemoradiotherapy (cCRT) followed by nivolumab plus ipilimumab compared to cCRT followed by durvalumab in participants with untreated, unresectable Locally Advanced Non-small Cell Lung Cancer (LA NSCLC). **Secondary Objectives:** To evaluate Overall Survival (OS), PFS between other study arms (Arm B vs Arm C and Arm A vs Arm B), Objective Response Rate (ORR), time to response, duration of response, time to death or distant metastases, and safety/tolerability.

2.2 Background & Rationale While concurrent chemoradiation (cCRT) followed by durvalumab (the PACIFIC regimen) is the current standard of care for stage III unresectable NSCLC, recurrence and death rates remain high, indicating a critical unmet need for improved long-term outcomes. Pre-clinical and early clinical data (such as the NICOLAS study) suggested that initiating immune checkpoint inhibition *concurrently* with cCRT, rather than sequentially, and adding a CTLA-4 inhibitor (ipilimumab) to a PD-1 inhibitor (nivolumab) during the consolidation phase might act synergistically to further improve patient survival and limit distant metastases.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Durvalumab) **Blinding:** Open-label
Randomization: 1:1:1 across three treatment arms

3.2 Planned Enrollment & Duration Targeted Enrollment (\$N\$): 925
Number of Sites: Global multicenter trial **Estimated Study Start:** August 20, 2019 **Estimated Primary Completion:** March 22, 2024 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years, male and female. 2. Newly diagnosed and treatment-naïve, with no prior local or systemic anticancer therapy given as primary therapy for locally advanced disease. 3. Locally advanced stage IIIA, IIIB, or IIIC (T1-2 N2-3 M0, T3 N1-3 M0, or T4 N0-3 M0) pathologically-confirmed NSCLC, according to 8th TNM classification. Participants who are not planned for potential curative surgical resection are eligible. 4. Eastern Cooperative Oncology Group (ECOG) performance status $\leq$ 1. (*Other protocol-defined inclusion criteria apply*).

4.2 Exclusion Criteria 1. Any condition including medical, emotional, psychiatric, or logistical that, in the opinion of the Investigator would preclude the participant from adhering to the protocol or would increase the risk associated with study participation. 2. Prior thoracic radiotherapy or prior local/systemic anticancer therapy given as primary therapy for locally advanced disease. 3. Active infection requiring systemic therapy within 14 days prior to randomization. 4. History of organ or tissue transplant that requires systemic use of immune suppressive agents. (*Other protocol-defined exclusion criteria apply*).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Participants are randomized into one of three arms: * **Arm A (Experimental):** Nivolumab plus cCRT followed by consolidation therapy with Nivolumab plus Ipilimumab. * **Arm B (Experimental):** Nivolumab plus cCRT followed by consolidation therapy with Nivolumab alone. * **Arm C (Active Comparator):** cCRT followed by consolidation therapy with Durvalumab.

Investigational Product (IP) Details: * **Drug Name / Preferred Name:** ipilimumab * **Trade Names:** Yervoy * **ATC Code:** L01FX04 * **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf

Dosage/Frequency: Intravenous (IV) infusions. Chemotherapy regimens depend on tumor histology (e.g., etoposide/cisplatin, paclitaxel/carboplatin, or pemetrexed/cisplatin). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Concurrent chemoradiation phase followed by an up to 1-year consolidation phase.

5.2 Concomitant & Prohibited Therapy Permitted: Protocol-specified platinum-doublet chemotherapy and thoracic radiotherapy (cCRT). **Prohibited:** Systemic immunosuppressive medications; prior CD137 agonists or other immune checkpoint blockade therapies outside of protocol assignment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Informed Consent, PD-L1 IHC 28-8 testing, Imaging, Medical History, ECOG assessment
2	Weeks 1-6	Randomization, cCRT administration ± Nivolumab, Safety Labs
3	Up to 1 Year	Administration of Nivolumab ± Ipilimumab OR Durvalumab, Efficacy Assessment (RECIST v1.1)
4	Post-treatment	Final Vital Signs, OS tracking, Exit Interview, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Arm A Vs Arm C - Progression-Free Survival (PFS) by RECIST 1.1 Per Blinded Independent Central Review (BICR):

Progression-Free Survival (PFS) is defined as the time between the date of randomization and the date of first documented disease progression, based on BICR assessments (per RECIST v1.1), or death due to any cause, whichever occurs first based on Kaplan-Meier estimates. *Note on Progressive Disease (PD):* At least a 20% increase in the sum of diameters of target lesions, taking as reference the smallest sum during the study (this includes the baseline sum if that is the smallest on study). In addition to the relative increase of 20%, the sum must also demonstrate an absolute increase of at least 5 mm.

7.2 Secondary & Exploratory Endpoints

1. Overall Survival (OS).
2. Arm B Vs Arm C and Arm A Vs Arm B - Progression-Free Survival (Irrespective of Subsequent Therapy) by RECIST 1.1 Per Blinded Independent Central Review (BICR).
3. Objective Response Rate (ORR) by BICR, time to response, and duration of response.
4. Safety profile and incidence of Grade ≥ 3 pneumonitis.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during the study period, including assessment of immune-mediated adverse events (imAEs) and chemoradiotherapy toxicity (e.g., pneumonitis) using CTCAE criteria. **Serious Adverse Events (SAEs):** Reported within 24 hours to the sponsor and Institutional Review Boards. **Data Monitoring Committee (DMC):** External independent Data Monitoring Committee assessing periodic safety and efficacy readouts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy analyses (PFS and OS). Safety Set for all patients receiving at least one dose of study drug. **Sample Size Justification:** 925 patients enrolled and randomized 1:1:1 to provide adequate statistical power ($\alpha = 0.05$) to detect a significant difference in PFS and OS between Arm A and Arm C. **Handling of Missing Data:** Censoring rules applied for time-to-event endpoints (PFS/OS) as per standard Kaplan-Meier methodology.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of trial sites to ensure protocol adherence, drug accountability, and accurate safety reporting. **Audit Trail:** Continuous eCRF monitoring, imaging quality control via BICR, and data clarification forms.

11. Study Identifiers & Governance

Posting ID: 939423464581887808 **Primary ID:** CA209-73L **Registry Number:** NCT04026412 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 73L **Criteria Type:** PHASE_REQUIREMENT (Criteria Text: Trial must be in PHASE3) **Phase ID:** PHASE3 **Source Level:** 5

12. Clinical Framework & Terminology

Disease Areas: Non-Small Cell Lung Cancer (NSCLC) **Primary Condition Name:** Non-Small Cell Lung Cancer (NSCLC) **Preferred UMLS Name:** Stage III Non-small Cell Lung Cancer **MeSH Code:** D002275 **SNOMED ID:** 254635002 **SNOMED Hierarchy:** 254637007|Malignant neoplasm of lung|Malignant neoplasm of lung (disorder)

12.1 Diagnostic Reagent Classifications * Preferred Name: Non-Small Cell Lung Cancer (NSCLC) Diagnostic Reagents * **Semantic Type:** Indicator, Reagent, or Diagnostic Aid * **Definition:** Molecular assay reagents intended for use in identifying translocations (i.e., exchanges of chromosomal material) involving the anaplastic lymphoma kinase (ALK) gene at chromosomal location 2p23. Translocations involving ALK may result in the generation of fusion genes, which have been implicated in the pathophysiology of multiple cancers (e.g., anaplastic large cell lymphoma, non-small cell lung cancer, renal cell carcinoma). Multiple fusion partners for ALK have been identified in human cancers, including EML4, KIF5B, NPM1, SQSTM1, and TPM3. These reagents are intended to identify translocations involving the ALK gene and any gene fusion partner.

13. Study Procedures & Methodology

Management Pathway: First-line systemic consolidation strategy post-chemoradiation **Biomarker Protocol:** PD-L1 expression testing utilizing the validated Agilent/Dako PD-L1 IHC 28-8 pharmDx assay **Response Evaluation:** Tumor assessments using RECIST v1.1 criteria by Blinded Independent Central Review (BICR) **Quality Audit Mechanism:** Independent Radiologic Review and ongoing Pharmacovigilance

14. Observation & Follow-Up Schedule

Total Duration: ~1 year of active consolidation treatment + long-term survival follow-up **Onsite Visit Cadence:** Aligned with IV infusion schedules (Q2W/Q4W based on assigned arm) **Remote Monitoring Frequency:** Standard ongoing survival sweep post-progression **Checklist Review Interval:** Imaging assessments occurring every 8-12 weeks per protocol RECIST v1.1 guidelines

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Investigator Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	Joseph Fiore	_____	[DD-MMM-YYYY]					
Lead Statistician	[Statistician Name]	_____	[DD-MMM-YYYY]					